

FC-001: Sarah Chen — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	FC-001
Persona	Family Caregiver	Next Step	Feed into `/summarize-interview` with other FC interviews

SESSION DETAILS

Date	2026-03-28	Duration	45 min
Interviewer	PM Lead	Setting	Remote, camera on
Location	Video call (Sarah at her office)		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your role --- what does a typical week look like for you caring for your mother?

I work full-time as a marketing manager in the CBD. My mum, Lin, is seventy-eight and lives alone about forty minutes from me. I call her twice a day — eight AM and seven PM. I visit twice a week, usually Wednesday evening and Saturday morning. The rest of the time I'm trying to coordinate between her visiting nurse Maria, her GP, the pharmacy, and my brother David who lives interstate and mostly just sends money.

Q: Are you the primary contact, or are there other family members involved?

I'm it. David is in Brisbane. He means well — he transfers money every month for mum's expenses. But money doesn't check if she took her pills. Money doesn't drive over when she doesn't answer the phone. I don't resent him, exactly, but I do sometimes wonder what it's like to care about someone from a distance without the daily weight of it.

Block 1: Falls

Q: Walk me through the last time your mother had a fall, or you were concerned she might.

Four months ago. Two AM. Mum got up to use the bathroom and tripped on the hallway rug. She fell on her hip. She couldn't get up. Her phone was in the bedroom. The pendant alarm I bought her was in the kitchen drawer — she stopped wearing it after two days because, and I quote, 'it makes me feel like an invalid.' She lay on that hallway floor for five hours. Five hours. Until my seven AM call went unanswered. I called again at seven-fifteen. Nothing. I drove there in a panic. Found her on the floor, dehydrated, frightened, with a bruised hip. No fracture, thank God.

[Emotional note] Sarah's voice cracked. She paused, took a breath, and said 'Sorry' before continuing. This is clearly the defining event.

[Probe] What was the hardest part of that situation?

The five hours. Not the fall itself. The five hours where I was asleep in my bed and my mother was on the floor calling out and nobody heard her. I was forty minutes away. I could have been there in forty minutes if I'd known. But I didn't know. That's the thing that wakes me up at three AM now — the not knowing.

Q: Have you ever gotten a false alarm --- thought something was wrong and it wasn't?

Yes. About two months after the fall, mum didn't answer my evening call. I called three times. Nothing. I drove over — forty minutes, rush hour, I was a mess. She was at the neighbour's house having tea. Forgot her phone. I cried in the car outside her house for ten minutes before going in. She was annoyed that I 'made a fuss.'

Block 2: Medications

Q: Tell me about the last time there was a problem with a medication.

It's not one incident. It's constant. Mum takes five medications. I fill her pill organiser every Sunday. And every Sunday, something's wrong. The wrong day is empty, or two days are empty when only one should be. She can't tell me what happened. She gets confused, then she gets angry at me for asking.

[Probe] Have you had a situation where you weren't sure whether a dose was taken?

Every week. And here's the thing I only found out recently — she's been gaming the organiser. Last month I came over on a Wednesday and the Tuesday compartment was empty, like it should be. But something felt off, so I counted the remaining pills. The numbers didn't add up. When I gently asked her, she eventually admitted she'd moved pills from Thursday to Tuesday to make it look right because she'd forgotten Tuesday's dose. She said she 'didn't want me to worry.' She's a retired maths teacher. She's not confused about this — she's deliberately covering her tracks. She moved her walking shoes to a different spot so Maria would think she'd been walking. She's managing the data, not managing her health. And I have no way to know what's real.

[Emotional note] Sarah alternated between frustration and grudging admiration: 'She's smarter than the system I set up for her.'

Block 3: Communication & Escalation

Q: When something concerning happens, how does information get to you?

It doesn't, unless I find it myself. Maria visits once a week. If something happens Monday evening and Maria doesn't come until Thursday, I don't know for three days. I text Maria on WhatsApp sometimes but she's overloaded with fourteen patients and I can tell I'm adding to her burden. For a while I tried using a wristband app that tracked mum's step count and sleep. I was sending Maria screenshots every week. 'Her steps are down forty percent.' 'She slept less than four hours.' Maria eventually told me, very nicely, to trust the clinical team. She was right — the data was creating anxiety, not insight. A step count without clinical context is just a number that makes you panic. I stopped using it.

Q: What would you actually need instead of raw data?

A verdict. Not a graph. Someone — or something — telling me: 'Your mother's patterns are within her normal range this week. Nothing to worry about.' Or: 'Something changed. Maria is looking into it.' That's it. Not a step count. Not a heart rate graph. A verdict from someone who knows what the numbers mean.

Block 4: Tools & Workarounds

Q: What tools do you currently use to monitor or coordinate care?

Two phone calls a day. A paper medication chart on the fridge. A pendant alarm that lives in a drawer. A WhatsApp group with Maria and my brother that my brother barely reads. A Google Doc where I track GP appointments. And — this is the ridiculous one — I track my mum's electricity usage through the energy company app. If usage is abnormally low by nine AM, it means she hasn't turned on the kettle, which means she might not be up, which means I call. It's absurd that I'm using the power bill to check if my mum is alive.

[Emotional note] Sardonic laugh here, but the underlying fear was audible.

Block 5: Willingness to Prioritize

Q: How much time per week do you spend on coordination?

Eight to ten hours. Fourteen phone calls, two visits, texting Maria, managing the pharmacy, the GP appointments, the Google Doc. That's on top of my full-time job and two teenagers.

Q: You mentioned you pay for the pendant. Have you evaluated other tools?

The pendant is eighty-five dollars a month. Unused. I tried the wristband app — free, but the false alarm anxiety was worse than no data. I looked at a smart home system last year but the setup was ridiculous and the test alarms went off every time mum's cat walked past the sensor. We lasted a week. I would pay a hundred and fifty to two hundred a month without thinking for something that actually worked. My brother offered to split costs.

Wrap-Up

Q: Is there anything I didn't ask that's important?

Talk to my mum. She won't tell you what she tells me — actually, she tells me less than she tells Maria. But the real story is what she's hiding from all of us. The pills she moves around, the dizziness she doesn't mention, the GP appointment she cancelled because she didn't want them to 'find something.' She's not confused. She's strategic. And whatever you build has to account for that.

Q: Who else should I talk to?

My mum, Lin. Maria Santos, her nurse. Angela Morrison at the agency — I call her twice a week and I'm sure she dreads seeing my number. And any other family caregiver who's been through the false alarm cycle — where you had data and it made things worse, not better.

PART B — Filled Note Template

Participant ID	FC-001
Persona	FC
Date	2026-03-28
Duration (min)	45
Interviewer(s)	PM Lead
KEY JOBS	Keep mother safe while living 40 min away. Know immediately when something goes wrong. Coordinate between mother, nurse, GP, pharmacy, and brother. Manage her own guilt and anxiety about not being physically present. Decode what's actually happening vs what mum reports.
CURRENT SOLUTION	2x daily phone calls. Paper medication chart on fridge. Pendant alarm (unused, £85/mo). WhatsApp group (brother barely reads). Google Doc for GP appointments. Energy company app as activity proxy. Previously: wristband step/sleep tracker (abandoned — data without context created anxiety + nurse friction). "She's been gaming the organiser. She moved pills from Thursday to Tuesday to make it look right because she'd forgotten Tuesday's dose." "A stranger showed up at my mother's door and nobody told me. My mother was frightened." "It's ridiculous that I'm using the power bill to check if my mum is alive."
WILLINGNESS	8-10 hrs/week on coordination. £85/mo on unused pendant. Previously spent on wristband app (free but abandoned). Would pay £150-200/mo. Brother offered to split. Key criterion: clinically interpreted information, not raw data.

EMOTIONAL INTENSITY	Voice cracked describing the 5-hour fall. Cried in car after false alarm drive. Sardonic about electricity tracking but underlying fear was real. Alternated frustration and admiration about mum's pill gaming: 'She's smarter than the system I set up for her.'
ESCALATION BEHAVIOR	Calls twice daily. If no answer on second call, drives over (40 min). Called 000 once (false alarm — mum at neighbour's). Paralysed between 'overreacting' and 'under-reacting.' The one time she waited, mum was on the floor.
SURPRISE FINDING	DATA GAMING: Lin deliberately manipulates the pill organiser to fake adherence. Also moves walking shoes to fake activity. Sarah's raw data from wristband created friction with nurse Maria (10-15 min extra work per visit) — 'data parent' behaviour. Abandoned monitoring because data without clinical context made everything worse.
TOOL MENTIONS	Pendant alarm (\$85/mo, unused). Wristband app (abandoned). Smart home system (abandoned after 1 week, cat triggered sensors). Energy company app (proxy). WhatsApp. Google Docs. Paper medication chart.
REFERRAL	Lin Chen (mother, SR-001). Maria Santos (nurse, HCN-001). Angela Morrison (coordinator, CC-001). Other caregivers who had the 'false alarm cycle.'
FOLLOW-UP ACTIONS	[] Interview Lin Chen (SR-001) — the 'other side' of Sarah's stories [] Investigate data gaming as a design constraint [] Explore role-specific alert thresholds (clinical vs family)